Docetaxel 100 mg/m² · every 3 weeks

NCCP 00202 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your treatment and access plan

Docetaxel 100 mg/m² by IV infusion on Day 1 every 21 days, with dose changes when needed. The number of cycles follows benefit, tolerance and the agreed treatment plan.

STEP 1

Before treatment

FBC, kidney and liver tests before treatment and every cycle. Review infection, neuropathy, skin/nail toxicity and swelling before the next dose.

STEP 2

Plan venous access

A cannula, PICC or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

Docetaxel infusion is about one hour. Blood review, preparation, premedication and observation add to the visit.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before chemotherapy; at least 3 doses before the infusion. If missed, contact the team; any IV replacement is a consultant decision.

Sources: NCCP 00202 v8 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.